

INTERMEDIATE LEVEL

MULTI-COMPOUND

12-16 WEEKS

ADVANCED PCT

INTERMEDIATE ANABOLIC CYCLE BLUEPRINT

Second & Third Cycle Strategies — Stacks, Synergy & Smart Protocols

Test + Deca • Test + Anavar • Test + Tren • Full Protocols

ROYAL FITNESS CLUB — ANABOLIC SERIES

Anabolic Series Vol. 2 • Intermediate Edition • 2024

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■ Prerequisites for Moving to Intermediate

Requirement	Minimum Standard	Why It Matters
Completed beginner cycles	At least 1–2 clean beginner test-only cycles	Know how your body responds to testosterone
Recovery confirmed	Post-cycle blood work shows normal LH/FSH/T	HPTA must be healthy before next cycle
Training age	4–6+ years structured training	Must be able to maximise compound stimuli
Knowledge	Understand aromatisation, AI use, PCT fully	Intermediate compounds have more complex profiles
Body composition	10–14% body fat pre-cycle	Lower fat = less aromatisation, better results
Time off	Minimum time off = cycle length + PCT	'On as long as off' rule — allow HPTA recovery

■ Intermediate Stack Options Overview

Stack	Compounds	Goal	Cycle Length	Difficulty	Side Effect Profile
Stack A	Test-E + Anavar	Lean bulk / Recomp	10–12 weeks	Low	Mild — low androgenic, no progestin
Stack B	Test-E + Deca-Durabolin	Mass & strength	14–16 weeks	Medium	Water retention, Deca dick risk if Test/Deca ratio wrong
Stack C	Test-E + Trenbolone Ace	Recomp / Cut	10–12 weeks	High	Night sweats, aggression, tren cough, prolactin issues
Stack D	Test-E + Masteron Prop	Hardening / Cut	10–12 weeks	Low–Med	Androgenic — hair loss risk if prone
Stack E	Test-E + Primobolan	Lean gains / preserve	12–14 weeks	Low	Very mild — expensive but clean

■ Stack A — Testosterone Enanthate + Anavar (Lean Bulk / Recomp)

Compound	Dose	Frequency	Weeks	Half-Life	Purpose
Testosterone Enanthate	400–500 mg	2x/week (Mon/Thu)	1–12	4.5 days	Base anabolic hormone
Anavar (Oxandrolone)	40–60 mg	Daily (split AM/PM)	5–12	9 hours	Lean muscle, strength, fat loss
Anastrozole	0.5 mg	EOD (as needed for E2)	1–12	—	Oestrogen control
TUDCA/UDCA	500 mg	Daily	5–12	—	Liver protection (oral compound)
Nolvadex (PCT)	40/40/20/20 mg	Daily	Wks 14–17	—	HPTA restart

■■ Stack B — Testosterone Enanthate + Deca-Durabolin (Mass Builder)

Compound	Dose	Frequency	Weeks	Critical Rule
Testosterone Enanthate	500 mg	2x/week	1–16	Test MUST be higher than Deca dose
Nandrolone Decanoate (Deca)	300–400 mg	1–2x/week	1–14	Never run Deca without Test as base
Anastrozole	0.5 mg	EOD	1–16	Monitor E2 closely — Deca also aromatises mildly
Cabergoline	0.25 mg	2x/week	1–16	Controls prolactin — prevents Deca dick
Nolvadex (PCT)	40/40/20/20 mg	Daily	Wks 18–21	Wait 3 weeks after last Deca pin to start PCT

■■ **IMPORTANT:** Deca-Durabolin suppresses natural testosterone VERY strongly and has a long half-life (~6 days). If Test:Deca ratio falls below 1:1, 'Deca Dick' (complete erectile dysfunction) is a real risk. Always run Test at 1.5–2x the Deca dose.

■ Stack C — Testosterone + Trenbolone Acetate (Advanced Recomp)

Trenbolone is the most powerful anabolic steroid widely available. It is 5× more anabolic and 5× more androgenic than testosterone, does not aromatise, but raises prolactin and causes significant cardiovascular and CNS side effects. Not for first-time intermediates.

Compound	Dose	Frequency	Weeks	Notes
Testosterone Enanthate	200–300 mg	2x/week	1–12	Cruise-level Test to maintain function
Trenbolone Acetate	200–300 mg	EOD (every other day)	1–10	Short ester — EOD injections required
Aromasin	12.5 mg	EOD	1–12	Preferred AI with Tren — Nolv can worsen prolactin
Cabergoline	0.25–0.5 mg	2x/week	1–12	Critical — controls prolactin/progesterone issues
Vitamin B6 (P5P)	200 mg	Daily	1–12	Additional prolactin support
Nolvadex (PCT)	40/40/20/20 mg	Daily	Wks 14–17	Begin 3 days after last Tren Ace pin

Trenbolone Side Effect	Severity	Management Strategy
Night sweats	Very common	Light bedding, temperature control — unavoidable
Insomnia	Common	Magnesium glycinate 400mg, melatonin 0.5mg
Tren cough	Common	Inject slowly, change injection sites — short-lived
Aggression / mood changes	Common	Dose management — never exceed 400mg/wk as beginner
Prolactin elevation	Moderate risk	Cabergoline — mandatory on all Tren cycles
Cardiovascular strain	Significant	Cardio 3–4x/week, monitor BP and RBC
Hair loss acceleration	High (if prone)	Nizoral shampoo — cannot fully prevent on Tren
Reduction in aerobic capacity	Common	Tren notably reduces cardio endurance — accept it

■ Full 16-Week Intermediate Cycle Calendar (Test + Deca Example)

Week	Test-E	Deca	Anastrozole	Cabergoline	Blood Work / Notes
1	250mg Mon+Thu	—	0.5mg EOD if sides	—	Saturation phase — Deca starts Wk 2
2	250mg Mon+Thu	200mg Mon	0.5mg EOD	0.25mg Tue+Fri	Deca beginning saturation
3	250mg Mon+Thu	200mg Mon	0.5mg EOD	0.25mg Tue+Fri	Monitor for water retention
4	250mg Mon+Thu	200mg Mon	Adjust per E2	0.25mg Tue+Fri	Mid-month check-in
5	250mg Mon+Thu	200mg Mon	Stable dose	0.25mg Tue+Fri	Strength increasing
6	250mg Mon+Thu	200mg Mon	Stable dose	0.25mg Tue+Fri	Mid-cycle bloods — adjust if needed
7-10	250mg Mon+Thu	200mg Mon	Continue	0.25mg Tue+Fri	Peak anabolic window
11-12	250mg Mon+Thu	200mg Mon	Continue	0.25mg Tue+Fri	Assess body composition
13-14	250mg Mon+Thu	200mg Mon	Taper	0.25mg Tue+Fri	Last Deca injection on Wk 14 Thu
15-16	250mg Mon+Thu	STOP	Low dose AI	Continue Caber	Clearance window — last Test pin Wk 16
17 (PCT Wk 1)	—	—	Stop AI	Stop Caber	End-of-cycle bloods. Start Nolv 40mg
18–20 (PCT)	—	—	—	—	Nolv 40/20/20mg. Post-PCT bloods Wk 21

■ ■ Intermediate Cycle Nutrition Protocol

Nutrition Factor	Bulking Stack (Test+Deca)	Recomp Stack (Test+Tren)	Lean Bulk (Test+Var)
Daily Calories	TDEE + 500–700 kcal	TDEE ± 100 kcal	TDEE + 300–400 kcal
Protein	2.5–3.0 g/kg bodyweight	3.0–3.5 g/kg bodyweight	2.5–3.0 g/kg bodyweight
Carbohydrates	5–7 g/kg (clean sources)	2–4 g/kg (cycle timing)	4–5 g/kg
Fats	0.8–1.0 g/kg	0.8–1.0 g/kg	0.8–1.0 g/kg
Meal Frequency	4–6 meals/day	4–5 meals/day	4–5 meals/day
Pre-workout meal	Carb + protein 60–90 min before	Moderate carb + protein	Moderate carb + protein
Post-workout	40–50g protein + 80–100g carbs	50g protein + 60g carbs	40g protein + 70g carbs
Sodium management	Moderate — manage water retention	Low — critical for recomp look	Moderate

Supplement	Dose	Purpose	Priority
Whey Protein (isolate)	2–3 scoops/day	Hit protein targets	Essential
Creatine Monohydrate	5g/day	ATP resynthesis, strength	Essential
TUDCA / UDCA	500 mg/day	Liver support (if using orals)	Essential with orals
Omega-3 Fish Oil	4–6g EPA/DHA daily	Cardiovascular health, HDL support	Essential
Vitamin D3 + K2	5000 IU D3 + 100 mcg K2	Hormone synthesis, calcium metabolism	Essential
Zinc	30–50 mg/day	Testosterone support, immune	Essential
Magnesium Glycinate	400 mg/night	Sleep, muscle function, insulin	Highly recommended
CoQ10	200 mg/day	Cardiovascular support, mitochondrial	Recommended on Tren
NAC (N-Acetyl Cysteine)	600 mg/day	Liver antioxidant support	Recommended
Taurine	3–5 g/day	Prevents back pumps from Anavar	Needed with Anavar
Vitamin C	1000 mg/day	Antioxidant, collagen (joint support)	Recommended